

Policy on Quality Improvement Projects

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» plans and implements quality improvement projects using a structured and documented approach.

The organisation runs a minimum of 2 quality improvement projects per year. Projects are selected based on high-risk, high-volume or problem-prone processes, or based on relevant indicators, incident analysis or patient-reported data.

1. Purpose

The purpose of this policy is to establish a structured approach for planning, conducting, documenting and reviewing quality improvement projects.

This policy does not cover matters outside quality improvement projects.

2. Scope

This policy applies to quality improvement projects undertaken by «Hospital Name».

3. Policy standards

3.1 Planning and selection

The organisation shall run a minimum of 2 quality improvement projects per year. Projects shall be selected based on high-risk, high-volume or problem-prone processes, or based on relevant indicators, incident analysis or patient-reported data.

Each project shall have a definite purpose, and a defined beginning and end; it shall be a time-bound activity. The parameters under improvement shall be measured both at the beginning and at the end of the project.

3.2 Structured methodology

Projects follow a structured methodology. The organisation could use PDSA, Six Sigma, Lean, or another appropriate improvement methodology.

PDSA (Plan-Do-Study-Act) tests a change on a small scale, studies the result, then decides whether to adopt, adapt or abandon it. Six Sigma targets near-elimination of defects using the DMAIC cycle (Define-Measure-Analyse-Improve-Control). Lean focuses on removing waste and non-value-adding steps from a process. Alongside a methodology, the organisation uses appropriate tools depending on the project — for example root-cause analysis and fishbone (cause-and-effect) diagrams to find causes, Pareto analysis to rank causes by frequency, run charts and control charts to track a measure over time, and flow charts to map the process being improved.

3.3 Outcomes

Quality improvement projects are centred on six domains of healthcare quality: patient safety, cost-effectiveness, patient-centredness, timeliness, efficiency, and equity in healthcare — a project should show its intended impact on at least one of these.

Project outcomes shall be documented, including whether the project achieved its stated goal. Outcomes shall be communicated to relevant staff and, where applicable, the governing body.

3.4 Patient-reported outcome measures

The organisation shall have a mechanism to capture patient-reported outcome measures (PROMs). Measurement instruments that patients complete to provide information about aspects of their health status shall incorporate information relevant to their quality of life, including symptoms, functionality and physical, mental and social health. PROMs may be generic or disease-specific. The organisation shall use PROMs to support communication and decision-making between doctors and patients and to improve patient satisfaction with care.

4. Non-negotiable rules

1. Do not count a project toward the annual requirement unless the organisation has run it as a quality improvement project.
2. Do not select projects without considering the stated risk, volume, problem-prone, indicator, incident-analysis or patient-reported data.

3. Do not treat a project as complete without documenting its outcome and whether its stated goal was achieved.
4. Do not make PDSA, Six Sigma, Lean or another specific methodology mandatory.
5. Do not operate without a defined mechanism to capture patient-reported outcome measures.

5. What we do

5.1 Plan and select projects

The organisation plans and selects a minimum of 2 quality improvement projects per year. Projects are chosen based on high-risk, high-volume or problem-prone processes, or based on relevant indicators, incident analysis or patient-reported data.

Each project has a definite purpose, and a defined beginning and end; it is a time-bound activity. The parameters under improvement are measured both at the beginning and at the end of the project.

5.2 Follow a structured approach

Projects follow a structured methodology. The organisation could use PDSA, Six Sigma, Lean or another appropriate improvement approach. The choice of methodology is not mandated.

5.3 Document project outcomes

Project outcomes are documented on completion, including whether the stated goal was achieved. Outcomes are communicated to relevant staff and, where applicable, the governing body.

5.4 Capture patient-reported outcome measures

The organisation has a mechanism to capture patient-reported outcome measures (PROMs). Measurement instruments completed by patients incorporate information relevant to their quality of life, including symptoms, functionality and physical, mental and social health. PROMs may be generic or disease-specific. PROMs are used to support communication and decision-making between doctors and patients and to improve patient satisfaction with care.

6. Stop-work authority

6.1 Quality improvement project trigger

Fewer than two quality improvement projects completed in a year is CORE non-compliance. The organisation must undertake at least two quality improvement projects per year. Failure to meet this minimum constitutes the specified stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Leadership	Supports planning and implementation of quality improvement projects. Provides resources required for the structured approach.
Quality Coordinator	Coordinates quality improvement projects. Implements projects using a structured approach and documents outcomes. Maintains the project register and monitors the annual project count.
Responsible staff	Implement assigned quality improvement projects following the structured approach. Document project outcomes and communicate results as directed.

8. Quality monitoring

Monitoring area	What is monitored
Project count	Minimum 2 quality improvement projects undertaken per year.
Project selection	Projects selected based on high-risk, high-volume or problem-prone processes, or relevant indicators, incident analysis or patient-reported data.
Methodology	Structured documented approach used for each project.
Project outcomes	Outcome documented for each project, including whether the stated goal was achieved.
Goal achievement	Whether each project met its stated goal.
Communication	Project outcomes communicated to relevant staff and, where applicable, the governing body.
Patient-reported outcome measures	Defined mechanism to capture PROMs and use of PROMs to support communication and decision-making between doctors and patients.

9. Training and staff acknowledgement

Staff involved in quality improvement projects are trained as required to follow the structured approach used for the project.

Staff acknowledgement:

I have read the Policy on Quality Improvement Projects of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to staff involved in quality improvement projects.

Project outcomes should be communicated to relevant staff and, where applicable, the governing body.

11. Abbreviations

Abbreviation	Meaning
NABH	National Accreditation Board for Hospitals and Healthcare Providers
PDSA	Plan-Do-Study-Act
PSQ	Patient Safety and Quality Improvement

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required. Note: levels corrected from source draft — PSQ.4.b and PSQ.4.d are Achievement, not Commitment.

Objective Element	Level	Traceability to this policy
PSQ.4.a	CORE	Sections 3.1, 5.1 and 6 address the minimum of 2 quality improvement projects per year, time-bound with pre/post measurement, project selection criteria, and the specified CORE stop-work trigger.
PSQ.4.b	Achievement	Sections 3.2 and 5.2 address the structured methodology for projects. The six health-care quality domains are the selection frame for the structured approach.
PSQ.4.c	Commitment	Sections 3.3 and 5.3 address documentation of project outcomes and whether the stated goal was achieved, using appropriate analytical tools as part of the

		structured approach.
PSQ.4.d	Achievement	Sections 3.4 and 5.4 address the organisation's mechanism to capture patient-reported outcome measures, the classification of measurement instruments as generic or disease-specific, and the use of PROMs to support communication and decision-making between doctors and patients.

13. Required Records/Evidence Checklist

- Annual quality improvement project plan showing a minimum of 2 projects per year.
- Project selection documents showing consideration of high-risk, high-volume or problem-prone processes and relevant indicators, incident analysis or patient-reported data.
- Project documentation showing the structured approach used.
- Completed project documentation showing the outcome and whether the stated goal was achieved.
- Project outcome communication to relevant staff and, where applicable, the governing body.
- Documentation of the organisation's mechanism to capture patient-reported outcome measures (PROMs).
- Description of the measurement instruments used, whether generic or disease-specific, and evidence that PROMs are used to support communication and decision-making between doctors and patients.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.4, "The organisation uses appropriate quality improvement tools for its quality improvement activities."

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